

REVISED VERSION

556 – OCTOBER 2008

LIFE AND DISABILITY CLAIMS QUESTIONS AND MARKING SCHEME

1.	(a)	<i>Outline what a Power of Attorney is and how such power is granted.</i>	(4 marks)	
	(b)	<i>Give one example of the circumstances when it might be used.</i>	(2 marks)	
	(a)	Power of Attorney (POA) gives an individual the legal power to act for another.		3
		All Powers of Attorney must be made by deed.		1
	(b)	PoA is usually granted where an individual is unable to manage their own affairs – e.g. when suffering from a cognitive impairment or out of the country.		2
				6
		Syllabus item: 5.6		
		Coursebook ref: 2C1 5F		
2.		<i>What information is contained in a life assurance policy schedule?</i>		
		A policy schedule will contain the individual details for the contract including:		
		• policy number;		1
		• details of life assured;		1
		• any special conditions;		1
		• policy start date;		1
		• policy type;		1
		• policy term;		1
		• premium amount and frequency;		1
		• the sum assured.		1
				6 max
		Syllabus item: 2.1		
		Coursebook ref: 1E2		

3. (a) *Why were extended headings for the definitions of critical illness introduced in the Association of British Insurers (ABI) Standard of Best Practice for Critical Illness in 2006?* (2 marks)

(b) *Give two examples of extended headings.* (4 marks)

(a) The ABI introduced extended headings for the definitions of critical illnesses to improve clarity about what is, and is not covered and to facilitate comparisons.

2

(b) Examples include – Heart Attack of specified severity, Cancer excluding less advanced cases, MS with persisting symptoms.

4

Examiners note – refer ABI documents for range of possible answers.

Syllabus item: 6.5

Coursebook ref: 7E

4. *There are two specific exclusions under the latest Association of British Insurers (ABI) definition of benign brain tumour. What are they and why are they excluded?*

Tumours in the pituitary gland are excluded due to its location and the contention that the pituitary gland does not actually form part of the brain.

3

Angiomas are excluded as they are benign growths that consist of blood vessels that rarely result in permanent neurological deficit.

3

6

Syllabus item: 10.1

Coursebook ref: 9A3

5.	(a) <i>What is dialysis?</i>	(2 marks)		
	(b) <i>What are the two main ways in which this is performed?</i>	(4 marks)		
	(a) Dialysis refers to a process whereby the filtering of toxic materials out of the blood, which is normally accomplished by the kidneys, is undertaken by some artificial means.		2	
	(b) In haemodialysis blood is diverted from the body and filtered through a kidney machine.		2	
	Alternatively the process can be by one or other of the forms of peritoneal dialysis. This is where fluid is inserted into the abdominal cavity to allow filtration to take place from the blood across the peritoneum which lines the abdominal organs.		2	4
	Syllabus item: 10.1 Coursebook ref: 8D3			
6.	(a) <i>Explain how a pregnancy exclusion clause operates in respect of an income protection claim.</i>	(4 marks)		
	(b) <i>What factors need to be addressed when drafting such clauses?</i>	(2 marks)		
	(a) Pregnancy clauses make it clear that pregnancy per se is not a valid cause for claim – and that incapacity must be related to illness or injury, typically some complication related to pregnancy.		2	
	Exclusion wordings vary across insurers according to what period the insurer will not consider a claim after pregnancy.		2	4
	(b) In 1998 a new exclusion clause was agreed by the Equal Opportunities Commission which allows for ‘complications arising from pregnancy, which are diagnosed as such by a doctor or consultant, who specialises in obstetrics’.		2	
	<i>Award 1 point for mentioning Sexual Discrimination Act and gender directives.</i>			
	<i>Examiner note - recent change of legislation here – not in text – give credit if more up-to-date info is supplied.</i>			
	Syllabus item: 5.1 Coursebook ref: 11I			

7. *When might a grant of representation be waived by an insurer settling a death claim?*

Sometimes a claimant may say that, as the value of the estate is so small, no grant will be applied for and the request is for the insurer to settle the claim without one. An insurer will not waive a grant when there is any dispute as to title.

2

Although there is no standard practice, many offices will pay claims under a certain limit without a grant. For example:

- sum assured up to £10,000; and/or
- Where the value of the estate is less than £25,000.

2

This will usually only be done where the payment is to a surviving spouse. It is usual for the life office to seek an indemnity from the claimant, ensuring an obligation for them to repay the sum if someone subsequently comes forward with good title. The approach taken by different life offices will to a large extent depend on their claims philosophies.

2

6

Points are also awarded for candidates who mention policies that were held under trust and where the life assured is domiciled abroad.

Syllabus item: 5.6
Coursebook ref:- 5A1

8. (a) *Briefly explain what disability living allowance is.*
(2 marks)

(b) *Outline the different components of the disability living allowance.*
(4 marks)

(a) Disability Living Allowance is a State Benefit payable to those under age 65 who are severely disabled and require assistance with the basic functions of life.

2

(b) There are two components.

The care component is for those who are severely mentally or physically disabled and who have needed constant care for at least three months.

2

The mobility component is for those whose principal disability is a lack of independent mobility.

2

4

Syllabus item: 4.5
Coursebook ref: 14B6

FOR USE WITH QUESTION 9 ONLY

STUDY THE INFORMATION BELOW AND THEN ANSWER THE QUESTIONS.

Policy Details	
Insured: Mrs Mary Stirling	Date of Birth: 13th August 1969
Term assurance with waiver of premium, critical illness and total and permanent disability.	Sum Assured: £250,000
Proposal date: 20 August 2006	Commenced 23 August 2006
Deferred Period: 4 weeks	Ordinary rates
Premium: £40 per month	Duration: To age 60
Proposal Form Summary	
Height: 165cm Weight: 65kg	Smoker: 20 cigarettes per day
Occupation: sales assistant – no manual duties	No adverse disclosures.
Definitions of Disability	
Total and Permanent Disability definition – totally and permanently unable, as a result of sickness or accident, to carry out any occupation for which he/she is reasonably suited by means of education, experience or training.	
Waiver of premium definition – totally unable as a result of sickness or accident to carry out the occupation notified to and accepted by us.	
Claim Form Information	
• Struggled for some months before finally having to cease work in February 2007. Have not been able to undertake any work since that time although did go back to place of employment on a couple of occasions purely to attend disciplinary hearings. • Suffering from a recurrence of Chronic Fatigue Syndrome. • Job involved a lot of travel, and carrying sample products from car to showrooms – up to 15kg each item. • Have been extensively investigated and treated by various specialists but now discharged back to GP. • Taking pain killers and homeopathic remedies. • Attendance at family outings and college lectures has been limited by pain and immobility. • Did apply for one job as a librarian but couldn't cope with lifting books so withdrew my application before the interview. • Have moved to a bungalow nearer to children's school to ease day to day living, and minimise child care burden. • 'I will never work again.'	

9. *Watter Life Insurance Company has been paying a waiver of premium claim in respect of Mrs Stirling for the past eighteen months.*

(a) *How would the claims assessor have established whether non-disclosure had occurred at the outset of the waiver of premium claim?* (10 marks)

(b) (i) *Outline how the claims assessor should undertake a review of the ongoing waiver of premium claim.* (18 marks)

(ii) *What different considerations would be necessary should a claim for total and permanent disability be received?* (22 marks)

Both parts of your answer to (b) should include reference to the specific issues that need addressing, along with the information and evidence that will be needed for the management of each claim.

(a) The possibility of non-disclosure should be considered in respect of all disability claims. The fact that this is a very early claim, for a subjective condition that is described as giving rise to symptoms some months before the date of disability in February 2007, raises significant possibility of non disclosure. Full medical records should be requested to see what if any consultations she had at the onset of her condition. Confirmation from her employers as to any time off should also be requested.

3

There is also talk of this being a recurrence of her condition and therefore the assessor would need to see whether earlier episodes should have been disclosed. The full medical records outlined above should demonstrate her past medical history, along with verification of other details such as smoking status, height and weight etc.

2

There also looks to be possible non-disclosure of her occupation – at proposal she described a non manual sales assistant role – and at claims stage the role is said to include significant driving and heavy lifting. Whilst the claims assessor will want to establish the true position here it is important to note that the WOP definition of disability means that the claim will be assessed against the occupation ‘notified and accepted’ by the insurer – therefore the manual aspects of the role can be disregarded when determining whether she is fit for her role.

2

However the assessor should still consider whether there has been non disclosure and if so what the appropriate remedy might be. For example if the underwriters would have increased premiums, or changed the definition of disability on TPD, or refused cover for WOP, had they been made aware of the full facts, then the policy may be compromised. The assessor must consider these aspects at the outset of the WOP claim – as they may be deemed to have waived their rights to do so when the TPD is considered at a later date.

1

Checks should of course be made to see if the policyholder did notify a change of occupational duties before the onset of the claim – and if so whether the insurer accepted these more manual duties. Clearly if this was the case then there is no consideration of non disclosure.

1

As with all non disclosure considerations, the assessor must review what was actually asked and stated at the proposal stage. If relevant questions were not asked then the policyholder cannot be expected to give details.

1

10

- (b) (i) The waiver of premium claim has been ongoing for 18 months, with a relatively low premium being waived each month. Evidence that would have already been obtained in this regard would include at least one claim form from the claimant and supportive medical evidence from her attending doctors.

1

The main ongoing requirement for the claims assessor is to establish that the claimant meets the definition of disability – namely that she is totally unable as a result of sickness or accident to carry out the occupation notified to and accepted by the insurer. As mentioned above – this would be the non manual sales assistant role mentioned on the proposal form – unless she had subsequently notified the insurers of a change, and they had accepted this.

2

The claimant will need to be asked to provide an updated account of her condition – to include symptoms, limitations, and what impact these have on her ability to undertake work. She would have had to give details of her insured occupation at the outset of the claim and the claims assessor will need to verify that her condition continues to prevent her from performing the associated demands of that occupation.

2

Medical evidence will be required on an ongoing basis. The evidence now required for assessment of a WOP claim will depend on what has been obtained to date – however an update from her regular GP will be required – along with any available specialist correspondence, since the last review.

2

If these do not enable a full assessment of whether the definition is met the claims assessor will need to consider other evidence tools such as an independent medical exam, home visit, surveillance etc.

1

For the WOP claim alone the sum assured does not warrant extensive evidence requests – although the claims assessor does need to satisfy themselves of definition fulfilment. However, they should in addition take into account any additional benefits on the policy such as TPD that might be claimed in the future.

3

It should be noted that the claimant had contemplated a role as a librarian. Although she decided against this due to the lifting involved her insured occupation is likely to be that of a sedentary nature and the assessor should therefore renew efforts to determine whether this lady is fit to undertake such a role. The claim form also seems to indicate that she is attending a college course – further details should be requested to see what the nature of this is, how often she attends, what her limitations are.

2

Her attempts to look for work, and further education are an encouraging sign and the assessor should capitalise on the claimant's motivation, and explore her circumstances further in this regard – either as the definition of disability may no longer be fulfilled – or to offer support and encouragement for a return to work in some capacity.

2

There is mention of a disciplinary hearing on the claim form and the assessor should consider if job dissatisfaction or poor performance are affecting her outlook. Job dissatisfaction can be a key barrier to successful recovery and return to work. If a claimant reports a history of significant job stress, work overload, performance issues, relationship breakdown or conflict with managers or peers in the workplace then the chances of successful return to work are seriously diminished unless these factors can be addressed in some way.

2

The claims management plan needs to reflect the need for ongoing supportive medical evidence, an exploration of her current circumstances, and ensuring that the WOP claims management complements the assessment of the TPD claim. In the meantime premiums should continue to be waived.

1 18

- (ii) The TPD claim needs to be assessed on the basis of whether the claimant is totally and permanently unable, as a result of sickness or accident, to carry out any occupation for which he/she is reasonably suited by means of education, experience or training. This introduces new assessment criteria that the claims assessor will need to consider.

1

Firstly the claims assessor needs to establish what roles the claimant is reasonably suited to. To do this they could send a questionnaire to determine all her relevant qualifications, previous occupations, hobbies, interests, and skills. This should give a reasonable idea of the types of roles that she could be considered for. This information may need to be supplemented by means of a home visit – or even a vocational assessment at a later date. It is worth noting that she is currently undertaking some form of college course – so the claims assessor could reasonably look to take in to account the skill/knowledge that she is acquiring at college at the present time. If the insurer is to argue that a claimant is suited to a role other than one they have previously undertaken they should be prepared to give examples of what type of role this might be.

3

When considering whether a claimant is totally unable to perform a range of occupations the assessor usually considers the key and material duties of such occupations, in order to make a reasonable recommendation.

1

Once the range of jobs to which she is suited has been established medical evidence needs to be sought – specifically determining her limitations and abilities – and how these impact on her ability to work. Given the sum assured in respect of this claim the assessor will need to ensure that all medical evidence is received – to include GP reports, specialist reports, nature of her homeopathic remedies, treatments and their efficacy to date. Given the nature of her condition the assessor will need to be aware of the variations of her ability on a day to day basis. If the information from her treating specialists is inadequate to determine her working capability – and note that she has been discharged by all specialists – an independent review from a suitably qualified person will be required.

2

It is always useful to obtain information about the claimant's day to day activities to understand the impact of symptoms. A home visit should be arranged and usually undertaken by a suitable health professional (mental health nurse, psychologist or occupational therapist with mental health experience). The visitor must have a good understanding of the condition, optimal recommended treatment; and understand the policy terms and conditions.

2

Activity diaries, observation of limitations, and specific medical questioning may help the claims assessor attempt to correlate the extent of incapacity with stated definition of disability.

2

Some claimants are reluctant to acknowledge that there might be psychosocial factors perpetuating their symptoms, so this needs to be dealt with sensitively if referral for psychological opinion occurs.

1

Liaison psychiatrist IMEs are valuable in cases where it is felt there may be co-morbid features i.e. anxiety, depression or sleep disorder that are contributing to the presentation and might require clinical management in their own right. Thorough consideration of the physical and psychological interplay and personalised treatment plan is required and this can be obtained from a liaison psychiatrist.

2

The fact that her specialists have discharged her may either mean that all treatment avenues have been explored and she is still so bad they can no longer help – or that other issues are underlying her incapacity and as such the medical profession are not able to assist further.

1

With input from a CMO, or specialist advisers, the claims assessor may seek to establish whether the claimant has undergone all the expected treatment for this condition – for example has cognitive behavioural therapy been considered/undertaken?

1

Even if the claims assessor feels that the definition of disability is met at the present time, the TPD definition requires that this is expected to be of a permanent nature. With a condition such as CFS this can be a very difficult decision, and the assessor will need to take advice from specialist CMO opinions who have access to full information, and who are satisfied that enough time has elapsed, and the appropriate medical intervention has been complete.

1

Permanent is deemed to mean- expected to last throughout the insured's life, irrespective of when the cover ends or the insured person retires.

1

As above, the issue of job dissatisfaction should be explored – with a disciplinary hearing at work part of her incapacity may stem from issues at work and these should be identified. Contact should always be made with the employer to ascertain any issues around performance, work relationships and support available to facilitate return to work.

2

The insurer may wish to fund CBT or GET (graded exercise therapy) but this should only be considered if the claimant is genuinely motivated, and there is a realistic prospect that such intervention will improve her condition to the extent that work can be considered. It can be helpful to engage the services of a vocational rehabilitation consultant to facilitate the return to work process in tandem with appropriate treatment.

2 22

Syllabus item: 6.1, 5.1

Coursebook ref: 20D, 16C

10. (a) *Describe the three components of a standard set of accounts.* (13 marks)

(b) *Explain the account entries that an income protection claims assessor will find particularly relevant for the assessment of benefit payable.* (21 marks)

(a) A standard set of accounts for any reasonably sized organisation will include three components:

- **Profit and loss account;** the profit and loss account presents a summary of the transactions of a business for a given period of time (normally but not always, a 12 month period)*. Accounts are prepared to ascertain the net profit or net loss in a business for an accounting period.

5

*This document shows turnover, cost of goods and services, and expenses.

- **Balance sheet;** the balance sheet is essentially a financial snapshot of the business at a single point in time, usually the final day of the accounting period. It represents the exact level of the business assets versus liabilities at that time. A balance sheet will indicate the financial strength of a business on the day that it is drawn up.

The balance sheet tells you:

- how much capital is in the business;
- the liquidity of the business (i.e. how quickly assets can be turned into cash);
- how solvent the business is (the likelihood it may go bankrupt);
- where the business sources its finance.

5

- **Statement of assets;** The statement of assets simply shows full details of the fixed assets summarised in the balance sheet. Individual property may be listed along with motor vehicles and office equipment or tools. The value of the asset will be shown at the start of the accounting period and also at the end.

3

13

- (b) The P&L account will show the nature of the business entity e.g. sole trader, partnership, limited company etc. This is very important for the claims assessor to know when determining the income of a claimant.

1

Gross profit is the difference between sales and cost of sales. This is an important figure as it measures how much was actually made directly from the business operations, before it starts to pay for overheads. The gross profit of the business is an important factor when considering the past performance of a business and comparing its financial position.

1

Net profit/loss represents the unadjusted profit or loss generated by the business after deducting all expenses from the gross profit figure. This figure must be adjusted for certain expenses disallowed for tax purposes e.g. depreciation to arrive at taxable profit.

2

In the case of the self-employed, the net profit of the business will be adjusted for depreciation, capital allowances and disallowable expenses and then represents the taxable income of those who operate that business. This may be a single individual but can also be a partnership.

2

In the case of a limited company, net profit (after adjustments) is not what is declared by the individual (who is a director of this company) in their individual tax return to Her Majesty's Revenue and Customs (HMRC). This usually consists of salary and dividends.

2

Each set of accounts must be looked at individually but some other common items of interest to an IP claims assessor include:

Do the account entries reflect the stated occupation of the claimant? i.e. are the expenses consistent with the role that the company is purported to be performing

1

Motor expenses can give an indication as to how much driving is involved.

1

Businesses are allowed to include as an expense the use of the home as an office. If such an item appears, it may enable the claimant to return to work faster than if they had to travel. Also, if there are any concerns over the claim then a claims visit may identify any signs of ongoing work.

1

If a wages figure is shown this may indicate that there are employees who may be keeping the business going in the absence of the claimant. If so, continuing income will need to be explored.

1

A figure for depreciation will usually be included in a set of accounts. This is not technically an expense. It represents the loss in value that the business assets will experience over the period covered by the accounts. This figure can be claimed against taxation but it must be reasonable for the HMRC to accept it. In practice the HMRC will add the depreciation figure back into the net profit.

1

When it comes to working out the tax assessment, the HMRC also allows a figure for what it describes as 'capital allowances'. Capital allowances are a percentage of the value of the asset deducted from the prior years figure. The percentages are stipulated by the HMRC. Depreciation and capital allowances, whilst similar in purpose, are usually applied at different rates, thus the amounts included can vary.

2

As it is not a true expense of the business, many insurers will allow depreciation to be added back into the accounts. But, if this is done, the capital allowances must be deducted from the revised net profit figure.

1

A limited company is owned by its shareholders and has to have at least one director. With larger organisations, there may be an unlimited number of directors. The director's share of the business should be stated in the accounts. Directors remuneration should also be included – although will not always be broken down by individuals.

2

Shareholders are remunerated by means of an increase in the value of the shares, or through dividends paid each year to the shareholders. A successful company will make a level of profit in any given year. Assuming no monies are retained in the business for future development or projects, then the remainder will be divided amongst the shareholders. So dividends can be treated as income for benefit calculations in some situations, but there are areas where caution must be exercised:

1

If dividends are to be taken as income, then they must be paid out of the adjusted net profit of the business generated in that year.

1

The organisation may continue to operate even when the policyholder is claiming a benefit. If this happens and net profit continues to be generated by the business, then the individual's share of the profit should be treated as 'continuing income' in any financial calculation subject to the exact wording of the policy definition.

1 21

Syllabus item: 4.3

Coursebook ref: 14D

11. (a) *What factors does a claims assessor need to take into account when considering the instruction of a Private Investigator (PI)?* (13 marks)

(b) *Outline the range and type of guidance that should be followed once the decision is made to instruct a PI.* (21 marks)

(a) Instructing a private investigator to investigate a claim should not be a routine occurrence. It should only be done where the circumstances of the claim merit it and alternative methods are not appropriate or have been exhausted and suspicion remains. The assessor needs to take into account the cost of the investigation versus the potential liability on the claim. 2

A private investigator can be appointed to investigate any particular claim type, some of the more typical scenarios being:

- a suspicious death overseas;
- evidence that a disability claimant may still be working despite assertions to the contrary from the claimant;
- where an assessor thinks evidence may have been fabricated to manipulate the claim. 2

Instructing a PI is an important step for an insurer and one that needs to be taken with care. It is essential that any PI appointed represents the insurer in a professional and ethical manner complying with all the relevant privacy and consumer protection legislation. 1

Most companies' claim philosophies will set out the circumstances in which consideration should be given to the appointment of a PI to investigate a claim. In addition it is increasingly common for companies to have their own template agreements to regulate the appointment. Any company claims philosophies and agreements should comply with the relevant industry guidance described below. 2

Prior to July 2007, the main guidance used by life and disability providers to determine whether or not to appoint a PI was informal guidelines drawn up by The Disability Service Providers Association (DISPA) and endorsed by the Health Claims Forum. That guidance covered the criteria to use when deciding whether or not to appoint a PI, how to select one that most appropriately suited your business needs and the code of conduct to be expected from them. 2

The Association of British Insurers (ABI) in conjunction with the Information Commissioner has also recently issued guidelines to the industry on the instruction of private investigators and tracing agents. The guidance was issued in

July 2007 and sets out best practice in the area. Although only voluntary guidance as such, it is normal for guidance from the ABI to be deemed to be the industry standard and non-compliance could impact on the way complaints are subsequently decided at the FOS.

2

One of the major factors for the previous guidance being updated by the ABI was the increasing impact of human rights legislation and the need to comply with a range of legislation designed to protect the rights of individuals – Human Rights Act, Data Protection Act Protection from Harassment Act.

1

In particular Article 8 of the Human Rights Act gives individuals the right to respect for private and family life – P.I.s must adhere to this and exercise discretion when considering pretext calls and taking photos of the insured in their home.

1

13

- (b) The ABI guidance sets out the following range of issues that need to be addressed when considering the instruction of a PI.

- Grounds for suspicion.
- Release of information to a PI to investigate the claim & instructing the PI.
- Information required from the PI to verify suspicion.
- Relationship between PI and insurer.
- Data protection considerations.

5

Grounds for suspicion

The insurer must have genuine reasons for believing that the claim is not genuine and these must be clearly recorded on the file. The grounds will vary depending on the claim type being considered. The instruction of a PI must be proportionate to the suspicion and if there are intermediate steps that can be taken in the first instance that should be done.

2

Information to be disclosed to the PI

Only the minimum necessary to allow the PI to perform their task should be provided to them. This must include sufficient information for them to be able to focus on the correct individual. The instructions should be explicit and clearly set out what is required of the investigators. Caution should be exercised in deciding whether or not to release details of a claimant's illness, in most cases it is considered sufficient for the insurer to generalise about the claimant's condition.

4

Instructing a PI

The insurer needs to know that any PI it appoints will act in accordance with the law and the relevant industry guidance as mentioned here. It is common for a written agreement to be drawn up setting out the basis of the agreement and the action authorised by the insurer. It is also increasingly common for PIs to have their own code of conduct setting out the standards that they will adhere to when investigating a case.

3

When selecting a PI, the insurer needs to consider:

- the skills and experience of the PI investigators;
- whether they hold and need any special licences;
- that the investigator holds appropriate indemnity insurance;
- that all data held will be in compliance with data protection legislation;
- the length of time the PI may need to hold any data they collect.

3

Information required from the PI to verify suspicion.

The insurer should only request the PI to obtain information that is reasonably necessary to establish the status of the claim. It should not request information that could only be obtained by deception or bribery, or is in breach of any privacy or harassment legislation.

1

Data protection considerations

There are data protection considerations for the PI, insurer and those connected with the insurer (reinsurer, legal and medical advisers) who might need to see the data.

1

The insurer needs to ensure it both retains and distributes data to interested parties in accordance with the Data Protection Act. This means it should consider the length of time it might need to hold data gathered by the PI, i.e. for no longer than is necessary for the purposes provided. It is suggested that holding such records for six years following the cancellation or lapsing of the policy in line with the Limitation Act would be an appropriate length of time.

2

21

Syllabus item: 3.4

Coursebook ref: 6F, 12I, 20Q1

12. ***What are the five possible cardiac or cardiovascular conditions or procedures covered by the Association of British Insurers (ABI) Statement of Best Practice for Critical Illness 2006?***

For each one, summarise the current definition and provide an overview of what constitutes a valid claim.

EXAMINER COMMENTS – note the definitions have been stated in full here – but candidates are only asked to summarise – and give an overview – points reflect this level of detail

There are five cardiac conditions/procedures that are covered by the ABI Statement of Best Practice for Critical Illness 2006. These are:

(1) **Aorta graft surgery – For disease**

The undergoing of surgery for disease to the aorta with excision and surgical replacement of a portion of the diseased aorta with a graft.

The term aorta includes the thoracic and abdominal aorta but not its branches.

For the above definition the following are not covered:

- Any other surgical procedure, for example the insertion of stents or endovascular repair.
- Surgery following traumatic injury to the aorta.

- A claim will be valid if surgery has been carried out to repair the aorta with a graft after it has been damaged through disease. This definition does not cover surgery to repair the aorta in other circumstances, for example, to repair damage caused by a traumatic injury.

(2) **Coronary artery by-pass grafts – with surgery to divide the breastbone**

The undergoing of surgery requiring median sternotomy (surgery to divide the breastbone) on the advice of a Consultant Cardiologist to correct narrowing or blockage of one or more coronary arteries with by-pass grafts.

Coronary artery by-pass surgery involves attaching a short length of blood vessel from another part of the body to by-pass a blockage in one or more of the coronary arteries that supply blood to the heart. Claims will only be valid if the by-pass procedure is done using surgery to divide the breastbone.

(3) **Heart attack – of specified severity**

Death of heart muscle, due to inadequate blood supply, that has resulted in all of the following evidence of acute myocardial infarction:

- Typical clinical symptoms (for example, characteristic chest pain).
- New characteristic electrocardiographic changes.
- The characteristic rise of cardiac enzymes or Troponins recorded at the following levels or higher
- Troponin T > 1.0 ng/ml
- AccuTnl > 0.5 ng/ml or equivalent threshold with other Troponin methods.

The evidence must show a definite acute myocardial infarction.

For the above definition, the following are not covered:

- Other acute coronary syndromes including but not limited to angina.

A heart attack occurs where the blood supply to the heart is interrupted, resulting in a portion of the heart muscle dying. The permanent damage to the heart muscle can be detected using an ECG machine which traces the heartbeat.

When someone has a heart attack, chemicals such as cardiac enzymes and troponins are released into the blood stream – these are usually present for several days after the event and can be detected by using a blood test.

At the time of having a heart attack, most people experience very severe chest pain and/or other symptoms such as sweating, vomiting, fainting and nausea. These symptoms can help doctors pinpoint precisely when the incident happened.

(4) **Heart valve replacement or repair – with surgery to divide the breastbone**

The undergoing of surgery requiring median sternotomy (surgery to divide the breastbone) on the advice of a Consultant Cardiologist to replace or repair one or more heart valves.

Having one or more defective heart valves replaced or repaired is covered if the procedure is done using open heart surgery involving the surgical division of the breast bone.

(5) **Major organ transplant**

The undergoing as a recipient of a transplant of bone marrow or of a complete heart, kidney, liver, lung, or pancreas, or inclusion on an official UK waiting list for such a procedure.

1

3

For the above definition, the following is not covered:

- Transplant of any other organs, parts of organs, tissues or cells.

A claim for major organ transplant will be valid in the event of heart replacement by undergoing a transplant. The claim will be valid from the point at which either the person is added to a UK waiting list for a suitable replacement organ to become available, or the organ transplant takes place.

2

34

Syllabus item: 10.1

Coursebook ref: 8 - various

13. (a) *What is the alimentary tract? Outline its main components and the functions performed by them.* (16 marks)

(b) *Provide an outline of the following two conditions.*

(i) *Crohn's disease.*

(ii) *Ulcerative colitis.*

(18 marks)

- (a)
- The alimentary tract begins at the mouth and ends at the anus. 1
 - Its function is to process food and liquid in such a way that the constituents can be easily absorbed into the body 1
 - In the mouth the act of chewing converts food from indigestible chunks into a pulp mixed with saliva, and the process of digestion is first initiated. 1
 - Swallowing transfers the food into the oesophagus, from where it passes into the stomach in the abdomen. 1
 - In the stomach there is a high concentration of acid produced by cells in its wall. Here the process of digestion and the splitting of food into simple components really begins. 1
 - Gradually the muscular wall of the stomach mixes and then propels its contents through its lower end, the pylorus which is a muscular valve, into the duodenum, the beginning of the small intestine. 2
 - The main role of the small intestine is absorption and its lining is especially adapted for this purpose. Absorption begins in the duodenum and continues throughout the next two parts of the small intestine, the jejunum and ileum. 2
 - Two important small ducts enter the duodenum soon after it begins; the first of these comes from the pancreas, which produces enzymes to help digestion. These enzymes are discharged through the pancreatic duct into the duodenum. 1
 - In addition certain specialised cells throughout the pancreas produce insulin which controls the level of sugar in the blood. 1
 - At the same site the common bile duct enters the duodenum. This carries bile, a liquid composed of bile salts and yellow bile pigment which is produced by the liver and collected by a network of tiny bile ducts which unite eventually to form the much larger common bile duct. 1
 - As the contents of the duodenum pass slowly down the length of the small intestine the foods which have been broken down into their simple constituents are absorbed by the lining of the intestine. These broken down constituents are carried in the veins of the intestine to the liver via the portal vein. In the liver they are processed into proteins, fats, carbohydrates and other essential materials. 2

- The last part of the intestine is the colon. This is shorter than the small intestine and its main function is to conserve fluid that is absorbed through its wall. Sodium is absorbed and potassium excreted to help regulate the body's electrolyte content. As the contents pass from the caecum they change from fluid to semi-solid faeces which are discharged from the rectum.

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- (b) (i)
- Crohn's disease and ulcerative colitis are inflammatory diseases of the bowel which may give rise to symptoms of diarrhoea with possible passage of blood rectally. They may also produce general symptoms such as weight loss and general ill-health.
 - Crohn's disease is of unknown cause in which there is chronic inflammation with associated swelling and partial obstruction to the small bowel. This commonly occurs at the terminal few inches of ileum, yet it also frequently affects the colon and rectum.
 - Crohn's disease gives rise to attacks of pain, partial or complete intestinal obstruction, and perforation of the bowel with the formation of an abscess. It usually begins in early adult life and is treated by drugs which suppress the inflammation. Of these prednisolone, a corticosteroid, is the most useful. Surgery may be required and usually involves resection of a length of diseased bowel and joining the ends together. The success rate is high but about a third of patients relapse and may require further surgery at some future date.
 - Abnormal communications between two parts of the bowel, or between the bowel and the abdominal wall or perineum may occur. These are normally an indication for surgical treatment.
 - Crohn's disease is serious and may be associated with manifestations outside the abdomen. These take the form of inflammatory changes occurring in the eyes (iritis), the joints (arthritis) or the skin (erythema nodosum). Occasionally carcinoma of the bowel may be a complication. The majority of individuals undergo remission either with surgical or medical treatment.

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(ii) Ulcerative colitis

- The cause of ulcerative colitis is unknown. The clinical features are due to inflammation of the lining of the colon which may undergo severe changes with widespread ulceration and a tendency to bleed. This gives rise to diarrhoea, often with very liquid stools containing blood and mucus.

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- The condition may be confined to the rectum where it is referred to as proctitis or it may to a variable extent involve the sigmoid, transverse or the right side of the colon to become 'total'.
- The condition is subject to remissions and exacerbations, which if severe may be life-threatening.
- This condition usually responds to medical treatment. Hospitalisation is often required and the diagnosis is confirmed by sigmoidoscopy (inspection of the rectum through a sigmoidoscope) or colonoscopy with biopsy of the lining of the bowel, which gives a precise diagnosis of the nature and extent of the inflammation.
- Lesions outside the alimentary tract, similar to those that occur in Crohn's disease, may be occasional complications.
- Ulcerative colitis usually responds to medical treatment but tends to recur and in severe cases it is necessary to perform an operation to remove the entire colon and rectum (proctocolectomy) leaving the patient with an ileostomy. This is a procedure where the terminal part of the ileum is brought out through the abdominal wall, and its contents are collected in a plastic bag which is emptied from time to time. A successful operation of this kind can allow the patient to remain in good health with a normal life expectancy. For those who retain the colon but who are subject to repeated attacks of colitis or chronic forms of the condition over many years, there is an increased incidence of carcinoma of the colon. This is especially true if its whole length is involved.

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Syllabus ref: 10.1

Coursebook ref: 17E

14. ***Explain the key components of a claim form for individual income protection. Outline the nature of information that should be requested in this document and why this information is needed.***

The claim form is a very basic, yet fundamental, part of the claims assessment process. It collects together the information that a claims assessor needs in order to commence the assessment process.

The construction of the form used by individual offices can vary. However, there are a number of common requirements in respect of personal identification, details of incapacity, employment status, financial position and a declaration. The wide range of information required means that income protection forms can be very detailed.

Personal identification - Much of the information is needed for basic identification, and to enable basic checks such as

- have premium payments been made at the right rate for age;
- are there any other policies held with that insurer – and with any others (checking on the ABI Health Claims Register);
- do any exclusions apply – eg some insured events may have age limits.

Typically this section will include policy number, full name and date of birth, contact details (by post, phone and e-mail if possible), and sometimes, marital status and number of dependants. Insurers may also ask about smoking status in this section.

Details of incapacity - The form must also elicit details of the claimant's incapacity. Claimants will be asked to give the nature of the condition(s) from which they are suffering and specifically how it limits their function. Details of medical advisers, along with treatment regimes and specialist referrals will also be required. A full and detailed description of the nature and severity of the symptoms being suffered along with any relevant medical history should be included. This will help the claims assessor start to understand the nature of the claim, and what form of claims management will be required – eg who to request further evidence from, and what level of information is required.

Where the incapacity was brought about by an accident this should be fully explained, along with the position of any compensation claim.

The claimant's own account of what prevents him/her from working, and when they hope to return to work are useful pieces of information for insurers to ascertain at an early stage.

Relevant dates must be included in order that the assessor can determine when the deferred period is to start, whether any linked claims could be considered, and what the financial position of the claimant as at the date of claim was. In addition, the possibility of non disclosure should also be considered once the claims assessor knows when the incapacity first arose.

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Employment status - It is important to obtain details of the claimant's occupation to check if any change has occurred from that described at proposal and to consider the impact of the alleged incapacity against the occupation described. In addition (depending on the definition of disability), alternative occupations should be considered.

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Both the job title and job duties should be requested, especially where the definition of disability relates to the claimant's own occupation.

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If the claimant is employed, the insurer may need to write to the employer for information concerning their occupation so full contact details should be requested

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Financial position – The claimant's employment status will decide on what financial evidence will be required - it is essential to obtain details of the claimant's finances to ensure that the limitation of benefit clause can be applied. Whilst supporting documents will always be required, the claim form should ask for basic details in line with the limitation of benefit clause. This will usually mean asking for details of the salary or earnings in the 12 months prior to the onset of incapacity, State Benefits claimed or entitled to, other insurances, and any pension, or income that continues during the incapacity.

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Declaration – Every claim form will contain a declaration at the end, the aim of which is to provide the following:

- a statement that the answers given are accurate and complete;
- an authorisation to approach any doctor to obtain medical evidence in accordance with the Access to Medical Reports Act;
- an authorisation to approach any other person for relevant information;
- an authorization for the release of information relating to the claimant to relevant parties, including reinsurers, other insurers, the ABI, Inland Revenue, DWP and doctors and medical examiners working on behalf of the insurer.

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Syllabus item: 3.3

Coursebook ref: 12A

